Symptoms and Treatment of Overdose:
Signs and symptoms - Drowsiness, dizziness, nausea, vomiting, diarrhoea, rash, various blood dyscrasias.

Treatment: - Empty stomach by including emesis or by gastric lavage, followed by administration of activated charcoal. Monitor vital signs and institute supportive measures, as needed. Dialysis is of little value because the drug and its metabolites are firmly bound to plasma proteins.

Storage Condition:
Keep container tightly closed. Store in a dry place (below 30°C).
Protect from light.

Pack Size:
Plastic container of 60ml, 90ml, 100ml and 120ml.

Products Registration Number:
MAL1986045034AZ

ROYCE®

Product holder / Manufactured by:
ROYCE PHARMA MFG. SDN.BHD. (650435-X)
PT 1663, Nilai Industrial Estate,
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Revision date: 130417

ROYCE®

Mefa Suspension 50mg/5ml

Mefenamic Acid - Analgesic)

Presentation:
Milky white, banana-flavoured suspension.

Content:
Each 5ml contains: Mefenamic Acid 50mg.

Indication:
1. As an anti-inflammatory analgesic for the symptomatic relief of rheumatoid arthritis (including Still's Disease), osteoarthritis, and pain including muscular, traumatic and dental pain, headaches of most aetiology, post-operative and post-partum pain; pyrexia in children.
2. Primary dysmenorrhoea.
3. Menorrhagia due to dysfunctional causes and presence of an IUD when other pelvic pathology has been ruled out.

Pharmacological Information:
An analgesic with anti-inflammatory properties, and a demonstrable antipyretic effect. It has also been shown to inhibit prostaglandin activity. Mefenamic acid was found to inhibit prostaglandin synthesis and to compete for binding at the prostaglandin receptor site. Pharmacologic show Mefenamic acid did not relieve morphine abstinence signs in abstinent, morphine-habituated monkeys.

Mefenamic acid is absorbed from the gastro-intestinal tract. Peak concentrations in the circulation occur about 2 hours after ingestion. Mefenamic acid is extensively bound to plasma proteins. Approximately 50% of a dose may be recovered in the urine within 48 hours, mainly as conjugated metabolites.

Following a single 1 gram oral dose, peak plasma levels of 10 g/ml occurred in 2 to 4 hours with a half-life of 2 hours. Following multiple doses, plasma levels are proportional to dose with no evidence of drug accumulation. One gram of Mefenamic acid given four times daily produces peak blood levels of 20 g/ml by the second day of administration.

Following a single dose, sixty seven percent of the total dose is excreted in the urine as unchanged drug or as one of two metabolites.Twenty to twenty-five per cent of the dose is excreted in the faces during the first three days.

In controlled, double-blind, clinical trials, Mefenamic acid was evaluated for the treatment of primary spasmodic dysmenorrhea. The parameters used in determining efficacy included pain assessment by both patient and investigator; the need concurrent analgesic medication; and evaluation of change in frequency and severity of symptoms characteristic of spasmodic dysmennorhea. Patients received either Mefenamic acid, 500 mg (2 capsules) as an initial dose and began first. After three menstrual cycles, patient were crossed over to the superior to placebo in all parameters, and both treatments (drug and placebo) were equally tolerated.

Dosage and Administration:
Mefa preparations should preferably be taken with food. Adults (and elderly over 65 years): 500 mg three times daily.

Menorrhagia-to be administered on the first day of excessive bleeding and continued according to the judgement of the physician.
Dysmenorrhoe-to be administered at the onset of menstrual pain and continued according to the judgement of the physician.

Mefa should be used with caution in elderly patients suffering from dehydration and renal disease.

Children: It is recommended that children under 12 years of age should be given Mefa Suspension in the following dose regime:
Infants over 6 months-25 mg/kg of body weight in divided doses, or
6 months-1 years: one 5 ml spoonfuls
2-4 years: two 5 ml spoonfuls
5-8 years: three 5 ml spoonfuls
9-12 years: four 5 ml spoonfuls

Dose may be repeated as necessary, up to three times daily. Apart from the treatment of Still's Disease therapy should not be continued for longer than seven days in children.

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

Contraindication:
Hypersensitivity to mefenamic acid; Active ulceration or chronic inflammation of the upper or lower GI tract; History of bronchospasm; allergic rhinitis or urticaria precipitated by aspirin or other non steroidal anti-inflammatory drugs; pre-existing renal diseases.

Warning and Precautions:
Warning
Risk of GI Ulceration, Bleeding and Perforation with NSAID
Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Therapy in patients with a history of ulceration or chronic inflammation of the upper or lower GI tract should be closely supervised.

Mefenamic acid may prolong the prothrombin time; monitor the prothrombin time frequently in patients who are taking oral anticoagulants concomitantly.

In suspected biliuria cases an alternative diagnostic procedure such as the Harrison spot test should be performed because Mefenamic acid may produce false-positive reaction for urinary bile in the diazo tablet test.

If abnormal liver function tests or other manifestations of liver dysfunction arise during therapy evaluate the patient for evidence of a more severe hepatic reaction. Discontinue therapy if abnormal liver function test results persist or worsen, signs and symptoms of liver disease develop, or secondary manifestations (eg. Eosinophilia rash) occur.

Mefenamic Acid eliminated primary by the kidneys. It should not be administrated to patients with significant renal impairment.

Pregnancy and Lactation:
Pregnancy
Use during pregnancy only if clearly needed. Administration of this drug in late pregnancy is not recommended because of the cardiovascular effects of prostaglandin inhibitors.

Nursing mothers
Trace amounts of mefenamic acid may be present in breast milk, and transmitted to nursing mothers. Therefore, it should not be given to women who are breast-feeding.

Interactions With Other Medicaments:
Risk of GI ulceration may increase when administered with aspirin and other salicyclates corticosteroids, indomethacin, pheylbutazone, and oxyphenbutazone. Prothrombin time of oral anticoagulants may be increased.
Hypoglycemic effect of insulin may be decreased.

Side Effects:
Diarrhoea occasionally occurs following the use of mefenamic acid. Bronchospasm may be precipitated in patients suffering from, or with a previous history of bronchial asthing or allergic disease.

In some cases reversible haemolytic anaemia has occurred. Temporary, lowering of the white blood cell count, which may have been due to Mefenamic Acid has been reported. Blood studies should therefore be carried out during long term administration.

Drowsiness and dizziness have rarely been reported.